



MEDICAL Department
SIMHADRI

Note No :1



Ref:SMTTP/MEDICAL/2024-25/MEDICAL/87714

Date:

04/05/2024(12:03:35)

Subject:APPROVAL FOR LMI ON FIRSTAID CONTROL AND MONITORING

The LMI on Firstaid control and monitoring Ref no: LMI-OD-OPS-SAFT-004,Rev:00,Dated 27.10.2023 is reviewed and no changes envisaged on LMI and location of Firstaid box and custodian list attached.

Put up for approval for competent authority

Enclosures:

- 1.LMI on Firstaid control and monitoring 2024
- 2.OD on Firstaid control and monitoring 2024
- 3.LMI on Firstaid control and monitoring 2021.

Submitted by:

टी श्रीधर - वरिष्ठ चिकित्सा अधिकारी

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On:04/05/2024(12:03:35) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:2

Submitted by:

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On:04/05/2024(18:58:08) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:3

May please review

Submitted by:

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Note No:5

Review and comment, if any.

Submitted by:

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On:18/05/2024(18:36:12) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:6

First aid kits are not availble in LT, CHP, Gen/Lab areas. However, HIRA is available for all the activities.

Submitted by:

मकाला नागेश्वर राव - उप महाप्रबंधक

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On:20/05/2024(09:35:49) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:7

As per the mail communication from CMO and AGM EMD Dated: 03 rd February 2024,

"

As SWITCHYARD being a 24*7 operational control room, its custodian and designated person may be shifted to OPERATION from EMD.

LMI may be amended accordingly

"

Submitted by:

तेन्नेटि विजय कृष्ण - अपर महाप्रबंधक

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On:20/05/2024(18:19:13) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:8

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On:21/05/2024(18:16:11) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:9

Changes have been done.

Submitted by:

टी श्रीधर - वरिष्ठ चिकित्सा अधिकारी

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On:23/05/2024(09:48:45) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:10

Reviewed

May process further

Submitted by:

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On:23/05/2024(11:37:43) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:11

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On:23/05/2024(17:30:42) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:12

Reviewed

May process further

Submitted by:

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On:27/05/2024(18:13:35) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:13

Submitted by:

पीयूष श्रीवास्तव - अपर महाप्रबंधक

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On:27/05/2024(19:05:08) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:14

Reviewed and is being forwarded for further process.

Submitted by:

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On:28/05/2024(16:52:15) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:15

Submitted by:

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On:28/05/2024(18:52:39) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:16

Reviewed, May process further,

Submitted by:

आन्नप्रगद रमा कोटेश्वरराव - अपर महाप्रबंधक

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On:03/06/2024(17:25:08) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:17

Submitted by:

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On:11/06/2024(19:54:15) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:18

Submitted by:

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On:12/06/2024(09:52:10) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:19

May be approved.

Submitted by:

भक्त बत्सल पात्रा - अपर महाप्रबंधक

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On:12/06/2024(10:33:48) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:20

Please vet the formatting

Submitted by:

सुरेश जॉन डेविड - महाप्रबंधक

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On:13/06/2024(09:13:36) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:21

Submitted by:

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On:25/06/2024(08:19:24) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:22

It is requested to modify as per standard format of Simhadri LMI (Guidelines for Issue of LMIs as a Controlled Document) LMI/OGN/GEN/002 Rev:04 as uploaded on Simhadri EEMG LAN

Submitted by:

अजय गुप्ता - उप महाप्रबंधक

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On:25/06/2024(09:46:01) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:23

Modified and uploaded.

Submitted by:

टी श्रीधर - वरिष्ठ चिकित्सा अधिकारी

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On:30/11/2024(18:38:01) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:24

Please check, if the LMI attached is as per the standard format of Simhadri.

Submitted by:

नवीन किशोर - अपर महाप्रबंधक

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On:02/12/2024(11:06:32) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:25

LMI no. Sholuld mention Rev. 01, No color coding.

Submitted by:

अजय गुप्ता - उप महाप्रबंधक

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On:04/12/2024(11:18:55) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:26

Modified and uploaded.

Submitted by:

टी श्रीधर - वरिष्ठ चिकित्सा अधिकारी

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On:30/12/2024(10:17:12) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:27

please correct LMI Name on each page,Write suprceded doc. no. & rev. details

Submitted by:

अजय गुप्ता - उप महाप्रबंधक

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On:30/12/2024(15:58:45) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:28

Updated in LMI

Submitted by:

टी श्रीधर - वरिष्ठ चिकित्सा अधिकारी

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On:02/01/2025(11:47:18) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:29

This LMI will have Rev.01, Superseded by Rev. 0. Mention Rev. 01 on every page along with LMI no. & also mention superseded doc. no. Rev. 0 along LMI No.

Submitted by:

अजय गुप्ता - उप महाप्रबंधक

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On:03/01/2025(09:44:23) **Ref:**SMTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:30

LMI had been updated based on the comment.

Put up for approval

Submitted by:

टी श्रीधर - वरिष्ठ चिकित्सा अधिकारी

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On:10/01/2025(12:21:21) **Ref:**SMTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:31

Month Nov 24 should not mentioned in LMI No. on each Page header.

Submitted by:

अजय गुप्ता - उप महाप्रबंधक

Ajay Gupta - 008565 (DY. GENERAL MANAGER ,EEMG)
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On:11/01/2025(10:46:30) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:32

LMI had been updated based on the comment.

Put up for approval

Submitted by:

टी श्रीधर - वरिष्ठ चिकित्सा अधिकारी

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On:13/01/2025(10:19:51) **Ref:**SMTTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:33

Submitted by:

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On:13/01/2025(11:54:43) **Ref:**SMTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:34

Modified and uploaded.

Submitted by:

टी श्रीधर - वरिष्ठ चिकित्सा अधिकारी

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On:13/01/2025(12:04:18) **Ref:**SMTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:35

Submitted by:

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On:13/01/2025(12:09:18) **Ref:**SMTPP/MEDICAL/2024-25/MEDICAL/87714

Note No:36

A typo at Page-13 has been corrected and the revised file uploaded at Annexure.

LMI is in line with the OD.

May please be approved.

Submitted by:

नवीन किशोर - अपर महाप्रबंधक

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On:17/01/2025(12:16:40) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:37

Submitted by:

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On:18/01/2025(09:44:57) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:38

Submitted by:

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On:21/01/2025(15:24:04) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

Note No:39

Approved & Submitted by:

समीर शर्मा - मुख्य महाप्रबंधक

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On:21/01/2025(16:04:43) **Ref:**SMTTP/MEDICAL/2024-25/MEDICAL/87714

NTPC SIMHADRI

LOCATION MANAGEMENT INSTRUCTION

FIRST AID CONTROL & MONITORING

LMI-OD-OPS-SAFT-004Rev:01

JAN 2025

ApprovedforImplementationbyHOP (SIMHADRI)

LMIAPPROVALDOCUMENT

1	LMINAME	FIRST AID CONTROL & MONITORING
2	LMINo.	LMI/OD/OPS/SAFT/004Rev:01 November 2024
3	ReferenceDocument	COS-ISO-00-OD/OPS/SAFT/004FIRST AID CONTROL & MONITORING
4	SupersededDocument	LMI/OD/OPS/SAFT/004/REV. NO.: 0,November 2021
5	PreparedBy	Name:Dr Thandra Sridhar, Senior Medical officer
6	CheckedBy	Name: Dr.K.Vijaya, CMO
7	ReviewedBy	Name: Prasanta KumarJena GM (O&M)
8	ApprovedBy	Name: Sameer Sharma H O P (Simhadri)

INDEX

SL.No.	Particulars	Page No.
1.0	SCOPE	04
2.0	Superseded documents	04
3.0	Details of Revision	04
4.0	Introduction	04
5.0	Roles and Responsibility	04
6.0	Requirements	05
7.0	Records	08
8.0	Audit	09
9.0	Review	09
10.0	Annexures	10-15

TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

FIRST AID CONTROL & MONITORING

1.0 SCOPE

The NTPC Safety Policy requires stations to assess what could cause harm to people as a result of work activities and to ensure in place appropriate precautions. When timely incident reporting does not happen, incidents have higher chances of repeating and potentially cause bigger incidents. This LMI focuses on improving compliance in reporting first-aid incidents, continual improvement, reduction of incidents, and covers aspects of control and monitoring first-aid provisions.

2.0 SUPERSEDED DOCUMENTS

LMI/OD/OPS/SAFT/004/REV. NO.: 0, November 2021

3.0 REVISION DETAILS

Checklist incorporated.

Entire document redrafted as per Simhadri LMI standard format.

4.0 INTRODUCTION

Continual improvement is an essential aspect of the NTPC Simhadri Safety Framework. Reactive elements provide opportunities to improve. It is necessary to analyse such opportunities and consider which can or should be acted on. First-Aid incident is a classic case of reactive measures as it is a post facto action. This LMI covers aspects of control and monitoring first aid incidents, with the intention to improve the reporting or capture of incidents with the intent to reduce incidents.

5.0 ROLES & ACCOUNTABILITY

HOP Simhadri is accountable for ensuring that these guidelines are implemented without modification at their location.

Chief Medical Officer: shall ensure requirements for adequate and appropriate first-aid facilities are maintained and periodically inspected at the location; be accountable to ensure all first-aid kits have a single-point designated person/HOS.

TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

HR-Head is responsible to ensure any transfers are communicated to Location Medical Officer for updating the first-aid kit responsibilities.

Designated person/Head of Section shall be accountable for the contents of first-aid kits in his control.

Site Safety Coordinator is accountable to monitor that periodic site safety assessments and/or inspections cover all first-aid boxes and facilities; to raise suo moto incident reports, where required and to ensure that all first-aid incidents are recorded/captured in the central system.

6.0 REQUIREMENTS

6.1 Risk-based Provision of First-aid facilities

All locations are required to comply with generally similar but, with minor differences in statutory requirements regarding first-aid. The requirement is adequate and appropriate first-aid facilities should be available such that persons can be given immediate help if they are injured or, taken ill at work.

The first aid facilities should correspond to the hazards which can be reasonably expected to occur in the workplace. For example, the hazards and related first aid required would be different for offices (low-risk environment) than, those required for 'hot work in a confined space' (high-risk environment). The location HIRA (Hazard Identification & Risk Assessment) study shall be the basis for identifying the requirements for providing facilities for first aid like the numbers of first- aid boxes, location of first-aid boxes, contents of first-aid boxes, numbers and availability of first- aiders. The terms kits and boxes are used interchangeably and mean the same. In addition to the above, the following should be considered during an assessment of first-aid provisions: location & remoteness of the workplace; the distribution and composition of the workforce; and the past history of incidents and their type, location and consequences.

6.2 First-aid Kit

The contents of the first-aid boxes again are to be determined by the hazards expected at the work location. In addition, the following shall apply:

6.2.1 What a First-Aid Box Should Not Contain:

TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

A workplace first-aid box is not a generic medicine cabinet and accordingly, should not have contents that are not first-aid items like, paracetamol tablets, antacid medicines, etc. Such superfluous items in first-aid kits tend to increase self-medication at the work place which should be avoided. Where required, such medicines may be made available at locations manned by para- medics.

6.2.2What a First-Aid Box Should Contain:

In some locations, statutory provisions provide detailed lists of items that are to be provided by the Occupier in the first-aid box. It is to be noted that as long as well equipped fixed first-aid facilities like manned first-aid rooms, are available such statutory requirements are easily complied with. The area first-aid box should contain essential firstaid items that based on that area's hazards and the evidence of a system of control and its implementation as given subsequently under 6.3. Standard List of Items to be kept in First Aid box are listed in Annexure-1.

Apart from first-aid items, the kit must include a system of control in the form of a list of items that are provided in the kit; the name of the person/section responsible for the kit; items utilization record with date, name and quantity; last inspection date, and; the location medical emergency phone number.

6.3Control

Any first-aid box is required to be used only when an incident happens.

While the purpose of providing first-aid box is to ensure timely availability of first-aid in case of an incident, it is mandatory that such user of the first-aid item to report the incident to his EIC or, Site Safety Coordinator. When that does not happen, incidents have higher chances of repeating and potentially cause bigger incidents. Thus, close control of first-aid kit contents is a critical safety assurance measure to confirm compliance of employees and contractors towards incident reporting requirements.

All first aid provided at a fixed first-aid facility shall be recorded; sample record is given as Annexure-3.

When first-aid was applied using the area/portable first-aid kit: the person/section responsible for the kit shall update the control sheet (see Annexure-2) with; items utilized, date, name, quantity and confirmation that;

All first-aid facilities shall have designated person identified by name for control purposes. This includes first-aid boxes that shall have one person whose name shall be mentioned in the list included with the first-aid box as mentioned under

TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

6.2. Each such designated persons or HOD shall be accountable for the contents of first-aid box under his/her control.

6.4 System of Inspections

All first-aid facilities shall be included in periodic inspections.

The location medical officer shall organize for periodic inspections of the first-aid boxes which shall be carried out, at least once a month. Site Safety Coordinator is accountable First Aid Control and Monitoring to monitor that the site safety assessments / inspections cover all first-aid boxes and where available, the fixed first-aid facilities. A sample first-aid box control/inspection checklist is given as Annexure-3.

6.5 Monitoring & Inspection

The main focus of the inspection of first-aid kits should be on replenishment quantities and types. The inspection should meticulously check for available quantity against provided quantity for each item and reasons mentioned for the use of items from the kit.

For any first-aid kit, there are two types of replenishments: routine and non-routine. Routine Replenishments cover replenishments of items which were observed to be damaged or, have crossed the useful life usually given as the date of expiry. Such routine replacements are expected and need not be reported other than for procurement of replenishments purposes.

Non-Routine Replenishments cover items that have been used for other than routine replenishment purpose. Non-routine replenishment requirements come up mainly because of two possible reasons:

- ✦ Misuse of first-aid items for other than first-aid work or,
- ✦ First-aid incidents – either recorded or, not recorded officially. The inspectors should firmly focus on non-routine replenishments.

For first-aid cases, if the incident is reported formally, the same would be mentioned in the first-aid kit list as provided under 6.2.

The biggest risk is where the first-aid incident has happened and items used from the nearest first-aid kit but, the incident is not reported. This risk is

TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

considered high due to the fact that unreported incidents cannot be investigated formally or, informally by competent persons like HOS/Site Safety Coordinator. This can lead to repeat incidents that may end up with similar or more grievous injuries. This is the risk that the inspections of non-routine replenishments should focus on.

6.6 Accountability for Usage & Suo Moto Incident Report

Where the first-aid item is used but not accounted for by the way of written comment in the provided sheet, such usage shall be accounted for by the person responsible for the first-aid kit. Where a first-aid box is assigned to a section, the respective HOS shall be accountable for the usage of the contents.

Suo moto incident shall be raised by the Location Medical Officer on receipt of the first aid kit inspection report with unaccounted usage of first-aid items. The same shall be updated in the central system and included in the periodic reports of the Site Safety Coordinator. Investigation of such incidents shall be undertaken by the Site Safety Coordinator as advised by the HOP.

6.7 Continual Improvement

The incidents raised either voluntarily or, as suo moto reports during first-aid box inspections should be analysed to establish whether safe work procedures need to be amended or, the employee/contractor needs additional training.

Incident Recall exercise should be undertaken by the respective Area Engineer/HOS to get to the root of the potential problem before it gets a chance to repeat. Where appropriate revision/additional Hazard Identification & Risk Assessments may be carried out to mitigate such risks.

Site Safety Coordinator shall ensure findings of such assessments are captured in the centralized system; trends analysed to identify potential associated system weaknesses; discussed in periodic Safety Committee Meetings; action planned to rectify system weaknesses, and included in periodic functional reports

7.0 RECORDS

Completed First Aid facilities inspection sheets are part of internal inspections and retained for future reference by the location medical officer (on-rolls or, outsourced). Evaluation of the compliance with this Guidance shall be carried out at least once annually and can be part of a larger scope technical or other

TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

audits. The audit shall be a combination of verification of records, verification of implementation in the field and interviews.

8.0AUDIT

System: Auditors shall validate the evidence of a system as specified under 6.0. Records: Verification of availability of the records detailed under 6.0.

Field Audit Checkpoints: Auditors shall take a random sample of at least two first-aid kits from at least two different functions, and where applicable from different control rooms, to assess the implementation of this LMI.

9.0Review

CMO will review & take the approval of competent authority (HOP). The document will be reviewed on 2 yearly basis or earlier, if required

TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

Annexure 1 – standard list of items to be kept in first aid box

Sl	Name of Item	Size Specification	Quantity
1	Sterile Gauze dressing	4"x 4"	6
2	Sterile Gauze dressing	6"x 6"	6
3	Sterile surgical pad	Standard	4
4	Cotton Swabs	Small Size	20
5	Sterile Eye Dressing/Pad	Standard	4
6	Eye wash Cup	Standard	4
7	Adhesive Plaster	2 cm x 1 cm	2 rolls
8	Bandage rolls	10 cm	4 rolls
9	Bandage rolls	5 cm	4 rolls
10	Antiseptic solution (Savlon)	60 ml bottle	2
11	Anti-burn cream	Standard	1
12	Antibiotic cream/ointment	Standard	1
13	Splint	Various sizes	3
14	Triangular Bandage	Standard	2
15	First Aid Leaflet		1
16	Notebook (for registration of name)	100 pages	1

TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

Annexure 2 – first-aid register

NTPC -SIMHADRI:

First-Aid Register Location:

Date	Time	Name of Injured person, id number	Why the need for first aid?	First aid administered	Outcome ? returned to work •sent to doctor	First Aid Incident Reported to Site Safety Coordinator by email or phone Mention Date & Time of Reporting	Name & Signature of First-Aider

Certified that all the First-Aid provided and all the first-aid incidents have been reported to the location Safety Officer/Head.

Location Medical Officer:

Date:

Certified that the First-Aid register was inspected and all the first-aid incidents have been recorded and conveyed to location Head.

Location Safety Officer: Date:

TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

Annexure 3 - first aid box inspection record

FIRST AID KIT INSPECTION RECORD	
Date	1 Dec 21
First Aid Kit #	FK-0100
Location	CHP Control Room
Person Responsible for this First Aid Kit	Mr AAA
Contact in case of Medical Emergency	

#	Items	Expiry date	Provided Qty.	Qty. Item used by: id number, name & date	Incident reported name & date / Remarks	Available Qty. during inspection (to be filled during inspection)
1	Band Aid	Jan 2023	10	Xxx, P2546	YYY, 21Aug21	
				Ppp, Q1234	YYY, 23Aug21	
				Rrr, E4789	YYY, 27Aug21	7
2	2-inch bandage roll		10			10
3	4-inch bandage roll		10	Ppp, Q1234	YYY, 23Aug21	
				Rrr, E4789	YYY, 27Aug21	8

TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

4	Burnol	Feb 2025	1 tube			Full tube
INSPECTION						
Certify that all provided contents are accounted for.					All the above Incidents are recorded and updated to HOP.	
Inspected By : signature, name & date					Safety Coordinator Sign:	

Location: Custodian: Mobile No:

TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

No.:

Standard List of Items to be kept in First Aid box

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7	Adhesive Plaster	2 cm x 1 cm	2 rolls
8	Bandage rolls	10 cm	4 rolls
9	Bandage rolls	5 cm	4 rolls
10	Antiseptic solution(Savlon)	60 ml bottle	2
11	Anti burn cream	Standard	1
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TITLE	FIRST AID CONTROL & MONITORING
LMI No.	LMI-OD-OPS-SAFT-004 Rev:01

LIST OF FIRST BOX LOCATIONS & CUSTODIANS

S.NO	FIRST AID BOX LOCATION	CONCERNED AREAS HOD/HOS	DESIGNATED PERSON
1	H.R DEPARTMENT	HOD/ HR	SRIMANTHA MAHAPATRA
2	CGM OFFICE	HOD/HOP	SR ES TO HOP
3	SSC C&M OFFICE	GM /C&M/ CPG 3	AJITH SINGH
4	SSC F&A 2ND FLOOR	CGM (VPG)	M.N.P. SWAMY
5	IT DEPARTMENT	HOD / IT	SOMANATH ROUT
6	SWITCH YARD	HOD OPERATION	CONTROLLER INCHARGE
7	DM PLANT	HOD	V LALITHA DEVI
8	NTPC MAIN STORE	HOD STORES	YASH PAL
9	ICH MAIN CANTEEN	HOD , HR	SRIMANTHA MAHAPATRA
10	CENTRAL STORE	HOD STORES	Sumita babu
11	SANKALP BUILDING (MTP HALL)	HOD MTP	B NAGAMANI
12	CHP CONTROL ROOM-2	HOD FM	A R KOTESWAR RAO
13	CHP WAG TIPPLER	HOD FM	A R KOTESWAR RAO
14	TRACK HOPPER	HOD FM	A K BEHARA
15	FUEL BHAVAN & COAL BHAVAN	HOD FM	A R KOTESWAR RAO
16	FC DEPARTMENT	HOD FM	A K BEHARA
17	CISF FIRE STATION	HOD HR	SRIMANTHA MAHAPATRA
18	WEIGHING BRIDGE	HOD FM	A K BEHARA
19	R&D CONTROL ROOM	HOD FM	A K BEHARA
20	SAFETY PARK	HOD SAFETY	TUSHAR KANUNGO
21	UCB CONTROL ROOM 1	HOD OPERATION	SHIFT INCHARGE
22	UCB CONTROL ROOM 2	HOD OPERATION	SHIFT INCHARGE
23	SILO STAGE 1	HOD OPERATION	SHIFT INCHARGE
24	SILO STAGE 2	HOD OPERATION	SHIFT INCHARGE
25	FUEL OIL PUMP HOUSE	HOD OPERATION	SHIFT INCHARGE
26	FIRST AID CENTER AMBULANCE	HOD CMO	DR T SRIDHAR
27	HOSPITAL AMBULANCE	HOD CMO	DR T SRIDHAR